ASKAPI / ClaimsSync

Strategic Revamp, Pricing & Market Expansion

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1. Executive Summary

ASKAPI is a proprietary healthcare claims integration platform developed by the three-partner team under Kaaryaa GenAI Solutions. It automates the download, classification, and routing of insurance claims, resubmissions, and remittance files from the Abu Dhabi Department of Health (DoH) Shafafiya API portal, eliminating manual effort for healthcare facilities.

This document provides a comprehensive review of what was built and delivered, an honest assessment of current pricing against market value, a strategic roadmap to transform the product from a thick-client tool into a cloud-hosted SaaS platform, and a market expansion opportunity analysis across the Middle East.

Key Finding

The initial implementation was delivered at approximately INR 6,000/man-day — roughly USD 9/hour effective rate. Market-equivalent pricing for this category of healthcare API integration software in the UAE is USD 80–150/hour or AED 4,000–15,000/year per facility on a SaaS model. With 5–6 facilities already live, there is an immediate revenue recovery and re-pricing opportunity. The total addressable market in UAE alone exceeds 500 facilities.

1.1 What Was Delivered

The following capabilities were built, tested, and deployed to production in approximately 143 hours (17.9 man-days) across July–September 2024:

Capability	Description	Status
Shafafiya API Integration	Full SOAP/XML integration with Abu Dhabi DoH Shafafiya API for claim and remittance download	Live
Multi-Facility Support	Single installation supports multiple facilities via shafafiaapi.ini configuration	Live
File Classification & Routing	Automatic routing of Claims, Resubmissions (with <Resubmission> tag), and Remittance files to correct folders	Live
3-Hour Interval Chunking	History downloads split into 3-hour windows to handle API limits (v7e enhancement)	Live
Attachment Removal	Strips <Attachment> and <Observation> blocks from resubmission XML files (v8a enhancement)	Live
TOTP-Based Admin Auth	2FA using TOTP (pyotp) for new client onboarding — OTP shared with Saleem only	Live
Host-Locked Licence	Encrypted licence tied to client machine hostname with configurable expiry date	Live

Capability	Description	Status
Windows Task Scheduler	Automated daily execution via Windows Task Scheduler at 7:00 AM	Live
PowerBI Integration (indirect)	Downloaded files feed Saleem's existing PowerBI reporting stack	Live
Installation & User Guide	25-page step-by-step ASKAPI v0 Installation Guide delivered to Saleem	Live

IP Ownership Reminder

All source code, architecture, encryption logic, TOTP admin gate, and documentation produced for this project is the sole intellectual property of the three-partner team. The 2FA OTP mechanism is the primary technical lever that controls onboarding access. This must be protected formally before any revamp investment.

2. Pricing Analysis — What Was Charged vs. What It Was Worth

2.1 Initial Project — Actual Charge

The following summarises the effort and billing from the original ShafafiaAPI Automation implementation:

Phase	Effort (Hours)	Man-Days	Description
Planning & Requirement Analysis	20	2.5	Requirement gathering, analysis, documentation
Design	12	1.5	API connectivity exploration and architecture
Development	81	10.1	Core download engine, parsing, routing, auth, INI handling
Testing & Integration	18	2.3	Binary build, AnyDesk integration testing, runbook
Deployment & Maintenance	12	1.5	First client onboarding, 2-week post-go-live support
TOTAL	143	17.9	Full project delivery

Billing Metric	Actual Charged	Notes
Rate Applied	INR 6,000 / man-day	Equivalent to ~USD 72/MD or ~USD 9/hour
Total Billed (est.)	INR ~1,07,000 (USD ~1,290)	17.9 MDs at INR 6,000
New Facility Onboarding	2 MDs (often unpaid)	Per note in effort document — 'not getting paid'
Facilities Onboarded	5–6 facilities	Via Saleem as channel partner
Onboarding Revenue Collected	Partial / Nil	Informal arrangement, no formal billing

2.2 Market Benchmark — What This Should Have Cost

Healthcare IT API integration is a specialised domain. Below is a benchmark of comparable services in the UAE/GCC market and India IT export rates:

Benchmark	Rate	Basis
UAE/GCC Healthcare IT Consultant	USD 120–250 / hour	Local UAE market rate for healthcare IT integration
India IT Export (Healthcare Domain)	USD 40–80 / hour	India NASSCOM export benchmark for healthcare software

Benchmark	Rate	Basis
India IT Export (General Software)	USD 25–50 / hour	Standard offshore rate
SaaS Integration Product (Annual/Facility)	AED 5,000–15,000/year	Middle East healthcare software subscription
ASKAPI Charged (Effective Rate)	USD ~9 / hour	What was actually billed
ASKAPI — Fair Market Rate (India Export)	USD 40–60 / hour	Minimum this should have been billed at

Revenue Recovery Gap

At the minimum India IT export rate of USD 40/hour for 143 hours, the project should have generated USD 5,720 (INR ~4.75 lakhs). At a UAE healthcare IT rate of USD 120/hour, it was worth USD 17,160 (INR ~14.2 lakhs). The team delivered at roughly 22% of minimum fair value. This gap must be corrected in the revamp pricing model.

2.3 Current Revenue Position (Estimated)

Item	Current Status	Amount (Approx.)	Opportunity
Initial Implementation Fee	Collected	INR ~1,07,000	Baseline — cannot be recovered
New Facility Onboarding Fees	Partially collected	INR 0–24,000 (0–4 paid)	Formalise and enforce at INR 12,000/facility
Annual Subscription / Renewal	Not in place	INR 0	Highest priority — set up immediately
PowerBI / Reporting Module	No separate charge	INR 0	Bundle or add-on AED 1,200/year
Support Retainer	Ad hoc AnyDesk	INR 0	Formalise at INR 5,000–8,000/month

Immediate Action — Subscription Formalisation

Before investing in the cloud revamp, the team must convert existing 5–6 facilities to a paid annual subscription model. Even at a conservative AED 3,000/year per facility with a 3-facility average under Saleem, that is AED 9,000/year = INR ~2 lakhs/year recurring — before a single line of new code is written.

3. Recommended Pricing Model — Current & SaaS

3.1 Immediate Pricing (Before Cloud Revamp)

The following pricing should be implemented NOW for the existing thick-client model, effective from the next licence renewal cycle:

Tier	Item	Recommended Price	Frequency	Who Pays
Onboarding	New Facility Setup	AED 1,500 – 2,500	One-time	Saleem pays Kaaryaa
Onboarding	Password / Config Update	AED 200 per incident	Per event	Saleem pays Kaaryaa
Subscription	Annual Platform Licence (per facility)	AED 2,500 – 4,000	Annual	Saleem pays Kaaryaa
Support	AnyDesk Remote Support	AED 150/hour	Per incident	Saleem pays Kaaryaa
Support	Monthly Support Retainer	AED 500 – 800/month	Monthly	Saleem pays Kaaryaa

Saleem's mark-up to his end facilities is his commercial decision. Based on comparable UAE healthcare software, he likely charges AED 5,000–10,000 per facility per year — meaning the team's AED 2,500–4,000 wholesale price gives him a healthy 40–60% margin, which is fair for a reseller managing client relationships.

3.2 SaaS Pricing Model (Post Cloud Revamp)

Once the platform moves to Azure cloud hosting, a per-facility subscription model becomes possible. This is the target state:

Plan	Who Is It For	Price	Includes
Starter	Single facility, basic download only	AED 3,500/year	Daily auto-download, Claims + Remittance + Resubmission routing, Email alerts, Basic download log
Standard	2–5 facilities, full features	AED 2,800/facility/year	Everything in Starter + Dashboard access, Multi-facility view, Password update self-service, Priority support
Enterprise	6+ facilities or hospital group	AED 2,200/facility/year	Everything in Standard + SLA guarantee, Custom reporting, API access, Dedicated support slot
Reporting Add-on	Any plan	AED 1,200/facility/year	Embedded dashboard, Claim/Remittance analytics, PowerBI or custom charts
Dubai / DHA Module	Any plan adding Dubai	AED 800/facility/year add-on	Dubai Health Authority API adapter when available

3.3 Revenue Projection — 3-Year Model

Conservative projection assuming Saleem retains existing 6 facilities and adds 3 new per year, plus 1 direct channel emerging in Year 2:

Revenue Stream	Year 1	Year 2	Year 3
Existing Facilities — Subscription (6 × AED 3,500)	AED 21,000	AED 21,000	AED 21,000
New Facilities (Saleem) — 3/year × AED 2,800	AED 8,400	AED 8,400	AED 8,400
New Facilities (Saleem) cumulative base	9 facilities	12 facilities	15 facilities
Onboarding Fees (3 new × AED 2,000)	AED 6,000	AED 6,000	AED 6,000
Reporting Add-on (50% uptake × AED 1,200)	AED 5,400	AED 7,200	AED 9,000
Direct Channel (New — Year 2 onwards)	—	AED 15,000	AED 28,000
Support Retainer (AED 600/month)	AED 7,200	AED 7,200	AED 7,200
TOTAL ANNUAL REVENUE (AED)	AED ~48,000	AED ~65,000	AED ~80,000
TOTAL ANNUAL REVENUE (INR, 1 AED = 23 INR)	INR ~11 lakhs	INR ~15 lakhs	INR ~18.4 lakhs

Revenue Share — 3-Partner Model

On INR 11 lakhs Year 1 revenue: If split equally, each partner earns ~INR 3.6 lakhs/year (passive, post-setup). Anbu should receive a larger share (suggested 50%) given 90%+ of solutioning and coding was his contribution. Suggested split: Anbu 50%, Partner 2 25%, Partner 3 25%. This needs a written partnership agreement.

4. Cloud Revamp — Architecture & Roadmap

4.1 Why Cloud Now

The current thick-client model creates operational fragility and commercial risk:

Current Problem	Business Risk	Cloud Solution
Installed on client PC	PC failure = no downloads, client complains	Cloud-hosted, always on
Version updates require manual re-install	Version drift across 5–6 facilities	Single codebase, one deployment
Password changes need Anbu's intervention via AnyDesk	Operational dependency, no revenue for effort	Self-service credential update portal
Abu Dhabi local hosting requirement	India team cannot access directly without AnyDesk	Azure UAE North region (Abu Dhabi data residency)
No visibility between Anbu and facility	Saleem is the only interface, creates dependency	Saleem admin portal + optional facility dashboard
Licence is a local .ini file	Can be bypassed or duplicated	Cloud licence managed server-side

4.2 Target Architecture — Azure UAE North

The recommended stack for ASKAPI SaaS uses Azure with UAE North region for data residency compliance with Abu Dhabi/UAE healthcare regulations:

Layer	Azure Service	Purpose
Scheduling	Azure Functions (Timer Trigger)	Replaces Windows Task Scheduler — triggers per facility daily at configurable time
Processing	Azure Functions (Python)	Core download engine: call Shafafiya API, parse XML, route files
Queue	Azure Service Bus	Job queue per facility — ensures sequential processing without overlap
File Storage	Azure Blob Storage (UAE North)	Stores downloaded claims/remittance/resubmission files. UAE data residency.
Database	Azure SQL / PostgreSQL	Facilities table, credentials (encrypted), download job logs, audit trail
Secrets	Azure Key Vault	Stores encrypted facility passwords — no plaintext anywhere
Admin UI	React + Azure Static Web Apps	Saleem's portal: onboard facilities, manage licences, view download status

Layer	Azure Service	Purpose
Facility UI (Optional)	React + Azure Static Web Apps	Facility login: view downloaded files, download stats, basic reports
Reporting	PowerBI Embedded or Metabase	Rich analytics on top of metadata DB — connects to Saleem's existing PowerBI skills
API Layer	Azure API Management + FastAPI	REST API for Admin portal and Facility portal to interact with backend
Auth	Azure AD B2C	Multi-tenant authentication — Saleem admin role, facility viewer role

4.3 Phased Delivery Roadmap

Phase	Duration	Deliverable	Strategic Outcome
Phase 1 — Lift & Shift	2–3 weeks	Containerise existing Python code. Deploy as Azure Function. Azure Blob for files. Remove Windows scheduler dependency.	Thick client eliminated. Platform is cloud-hosted. Single version serves all facilities.
Phase 2 — Multi-Tenant DB	3–4 weeks	Introduce facilities table in Azure SQL. Credential management in Key Vault. Download job tracking and audit log. Saleem admin CLI or simple web form.	Full visibility of all facilities. Licence managed server-side. Credential updates without AnyDesk.
Phase 3 — Saleem Admin Portal	3–4 weeks	React admin portal for Saleem. Onboard facility, update password, view download status, trigger manual run, set expiry.	Saleem is self-sufficient. Reduces Anbu's operational involvement to near zero.
Phase 4 — Provider Abstraction + Dubai	2–3 weeks	Refactor Shafafiya calls behind HealthcareProvider interface. Implement Dubai Health Authority adapter when API specs available.	Single platform supports Abu Dhabi + Dubai. Foundation for Saudi, Qatar, Oman expansion.
Phase 5 — Facility Portal + Reporting	4–5 weeks	Facility login and dashboard. Download history, file counts, claim/remittance analytics. PowerBI Embedded or Metabase.	Facility managers have direct visibility. Upsell opportunity. Reduces Saleem's support calls.

4.4 Azure Cost Estimate (Monthly, at 10–15 Facilities)

Azure Service	SKU / Tier	Est. Monthly Cost (USD)
Azure Functions	Consumption plan (low volume)	USD 5–15
Azure Blob Storage	LRS, UAE North, ~50 GB	USD 5–10
Azure SQL	Basic tier (5 DTU)	USD 5–10
Azure Service Bus	Basic tier	USD 1–3
Azure Key Vault	Standard, ~100 operations/day	USD 1–2

Azure Service	SKU / Tier	Est. Monthly Cost (USD)
Azure Static Web Apps	Free tier	USD 0
Azure AD B2C	50,000 MAU free tier	USD 0
TOTAL ESTIMATED	10–15 facilities	USD 17–40 / month (INR ~1,400–3,300)

Cost vs. Revenue

At USD 30/month Azure cost (midpoint) against AED 48,000/year (Year 1 revenue), the cloud infrastructure cost is roughly 1.6% of revenue — extremely favourable for a SaaS business. The platform economics are very strong.

5. Saleem Admin Portal — Functional Specification

The Admin Portal gives Saleem a professional web interface to manage his client base without requiring AnyDesk or technical intervention from the development team. This is a critical deliverable for making Saleem a genuine, empowered reseller rather than a manual relay between facilities and the dev team.

5.1 Core Modules

Module	Key Features	Access Level
Dashboard	Active facilities count, Last successful download per facility, Failed jobs with reason, Files downloaded today/week, Renewal alerts (30/15/7 days)	Saleem Admin
Facility Management	Add new facility (Facility ID, User ID, Password, claims/remit/resub folder paths), Edit facility credentials (no plaintext display — write-only), Activate / Deactivate toggle, Set licence expiry date, Delete facility	Saleem Admin
Download Monitor	Per-facility download job history (date, status, files count), Trigger manual download for specific date range, View last 30 days of download logs, Export log to CSV	Saleem Admin
Billing & Subscription	View all facility subscription expiry dates, Send renewal reminder email to self, Generate PDF invoice stub, Mark facility as paid / unpaid	Saleem Admin
Platform Admin (Kaaryaa Only)	Create/revoke Saleem admin accounts, Platform-wide facility counts, Revenue dashboard, Audit log of all Saleem actions, OTP management for onboarding	Kaaryaa Super Admin

5.2 Saleem Portal — Workflow: New Facility Onboarding

1. Saleem receives client credentials from new facility
2. Logs into ASKAPI Admin Portal at askapi.kaaryaa.io (example domain)
3. Clicks 'Add New Facility' — enters Facility ID, User ID, Password, subscription end date
4. System encrypts credentials via Azure Key Vault. Folder structure provisioned in Azure Blob.
5. System triggers a test download to validate credentials. Result shown to Saleem.
6. Daily schedule activated automatically. Saleem receives confirmation email.
7. Kaaryaa receives auto-notification: new facility onboarded — invoice trigger.

Key Benefit

This flow eliminates 100% of AnyDesk sessions for routine onboarding. Anbu's time is freed from operational tasks and redirected to platform development and new market expansion. Saleem feels empowered and professional — he has his own branded tool.

6. Market Analysis — UAE and Middle East Opportunity

6.1 Abu Dhabi (Current Market)

Abu Dhabi is the team's established market through the existing Saleem channel. The opportunity is larger than the current 5–6 facilities suggest:

Market Metric	Data / Estimate
Licensed Healthcare Facilities in Abu Dhabi (DoH)	~300+ (clinics, hospitals, diagnostic centres)
Shafafiya API-connected facilities (mandatory for insurance billing)	Majority of licensed facilities
Facilities currently using manual/Excel-based claim management	Estimated 30–50% — highest pain point
Annual value per facility (ASKAPI subscription)	AED 2,500 – 5,000
Total Abu Dhabi TAM (300 facilities × AED 3,000 avg)	AED 900,000/year (~USD 245,000)
Realistic 5-year capture (5% = 15 facilities)	AED 45,000–75,000/year
Current ASKAPI position	5–6 facilities (Saleem channel) — ~2% penetration

6.2 Dubai (Next Priority)

Dubai Health Authority (DHA) operates a separate healthcare claims ecosystem from Abu Dhabi DoH. A Dubai-compatible ASKAPI module is the highest-value near-term expansion:

- Dubai has 150+ licensed private hospitals and 1,000+ licensed clinics as of 2024
- DHA operates its own eClaims portal and API — separate from Shafafiya but conceptually identical workflow
- Dubai is the team's cleanest path to a direct (non-Saleem) client relationship
- A second channel partner in Dubai (similar to Saleem's role in Abu Dhabi) is the recommended go-to-market
- Key difference: DHA API specs are publicly documented — no discovery risk, just adapter development

Dubai Market Metric	Estimate
Licensed healthcare facilities in Dubai (DHA)	~1,200+ (hospitals + clinics combined)
Target segment (mid-size private clinics)	~200–300 facilities
ASKAPI potential capture (3-year, 5% penetration)	10–15 facilities
Revenue at AED 3,500/facility/year	AED 35,000–52,500/year

Dubai Market Metric	Estimate
Development effort for Dubai adapter	~2–3 weeks (once architecture is modular)

6.3 GCC Expansion — 3-Year Horizon

The broader GCC market has comparable healthcare regulatory frameworks, all moving toward mandatory e-claims and API-based insurer connectivity. ASKAPI's architecture, once provider-agnostic, maps directly:

Country	Health Authority	e-Claims System	Market Size (Facilities)	ASKAPI Entry Timeline
Saudi Arabia	CCHI + SFDA	NPHIES (National Platform)	2,000+ licensed facilities	Year 2–3 (largest market)
Qatar	PHCC / NHIC	ClaimLink	200+ facilities	Year 2
Bahrain	NHRA	BUPA / insurance APIs	100+ facilities	Year 2–3
Kuwait	MOH Kuwait	Al-Dhaman (GOSI health)	300+ facilities	Year 3
Oman	MOH Oman	Daman / OMSB	200+ facilities	Year 3

Saudi Arabia — The Jackpot

Saudi Arabia's Vision 2030 healthcare privatisation drive has created 2,000+ licensed private facilities all mandatorily connected to NPHIES for insurance claims. A Saudi-adapted ASKAPI — called ClaimsSync KSA — could be the biggest single revenue opportunity in the GCC. Entry strategy: find a Saudi-based healthcare IT reseller (equivalent of Saleem, but in Riyadh or Jeddah). The product is already 80% applicable.

6.4 Competitive Landscape

The UAE and GCC healthcare IT integration market has some players, but no dominant low-cost automated download solution for small-to-mid facilities:

Competitor Type	Examples	Weakness ASKAPI Can Exploit
Large HIS Vendors (Epic, Cerner)	Epic UAE, Oracle Health	Priced at USD 50,000+/year — unaffordable for small clinics
Local EMR/PMS vendors	iClinic, Nabidh-integrated systems	Bundled with EMR; API download is an afterthought, not specialised
Manual/Excel processes	Most small clinics today	No cost to switch; ASKAPI is their first automation — easiest sale
Custom IT agencies	Various UAE IT firms	One-off projects, no SaaS model, high cost, no IP continuity

Competitor Type	Examples	Weakness ASKAPI Can Exploit
ASKAPI (us)	ASKAPI / ClaimsSync	Specialised, affordable, proven, SaaS-ready, UAE data-resident

7. Business, Legal & Partnership Structure

7.1 IP Formalisation — Critical First Step

Before any investment in the cloud revamp, the three-partner team must formalise intellectual property ownership. Currently the IP protection relies on an informal understanding and the TOTP admin gate. This is insufficient as commercial value grows.

Document Needed	Purpose	Priority
IP Assignment Agreement	Confirms all code, architecture, documentation is owned jointly by the 3 partners in agreed shares	CRITICAL — do this week
Partner Profit Sharing Agreement	Documents revenue split (recommended: Anbu 50%, Partners 2 and 3 at 25% each, reflecting Anbu's 90% contribution)	CRITICAL — do this week
Channel Partner Agreement with Saleem	Documents Saleem's role as authorised reseller, his pricing rights, territory (Abu Dhabi initially), and obligations	HIGH — before next renewal
Service Level Agreement Template	Standard SLA to attach to each facility subscription — defines uptime, support response times, data handling	MEDIUM — before SaaS launch
Non-Compete Clause (with Saleem)	Prevents Saleem from replicating the solution with another developer	HIGH — include in channel agreement

Protecting the 2FA Gate

The TOTP OTP for new client onboarding is currently the single most important technical IP protection mechanism. The secret key (IIBPORVRB5I6XVIB5C6C4RNQ5M3BQ7CJ) must remain known only to the three partners. In the cloud revamp, this becomes a server-side Kaaryaa Super Admin control, not embeddable in any client-side code.

7.2 Channel Partner Structure — Saleem Model

The recommended operating model positions Saleem as a commercial reseller with a defined, enforceable arrangement:

Aspect	Recommended Model
Saleem's Role	Authorised Reseller for Abu Dhabi (initial territory). Manages facility relationships, collects payments, provides first-level support.
What Saleem Gets	Reseller margin (difference between Kaaryaa wholesale price and his facility retail price). His suggested retail: AED 5,000–8,000/facility/year.
What Kaaryaa Gets	Platform wholesale fee: AED 2,500–4,000/facility/year, billed to Saleem — regardless of whether Saleem collects from his clients.

Aspect	Recommended Model
Billing Flow	Kaaryaa invoices Saleem. Saleem invoices his clients. Kaaryaa has no visibility or dependency on facility payment.
New Onboarding	Saleem pays Kaaryaa AED 1,500–2,000 per new facility onboarding. Kaaryaa activates the platform entry.
Territory Exclusivity	Saleem gets Abu Dhabi exclusivity for 2 years. Dubai and other emirates are Kaaryaa's direct territory unless agreed otherwise.
Renewal Control	Kaaryaa controls subscription expiry on the cloud platform. Non-payment by Saleem = facility deactivated from Kaaryaa side.

7.3 Go-Direct Strategy — When and How

Approaching Saleem's existing clients directly would damage trust and is commercially unwise. The go-direct strategy applies to NEW clients Saleem has never touched:

- Year 1: Saleem-exclusive. Build platform, formalise agreements, convert renewals to paid subscriptions.
- Year 2: Find a second channel partner in Dubai — different geography, no conflict with Saleem.
- Year 2–3: Direct outreach to Abu Dhabi facilities that Saleem has never engaged — estimated 290+ untouched facilities.
- Year 3+: Healthcare IT conferences, LinkedIn outreach to Practice Managers in UAE, referral programme.

Saleem's Motivation to Cooperate

Saleem has invested time and credibility with his clients to implement ASKAPI. A cloud upgrade makes his solution better, reduces his support burden, and increases his credibility. Frame the revamp as 'upgrading your clients to a professional SaaS platform' — Saleem should be enthusiastic, not threatened. His existing clients get a better product; he gets less support calls.

8. Implementation Plan — Next 90 Days

8.1 Immediate Actions (Week 1–2) — No Code Required

8. Hold a 3-partner call to review and agree on this strategy document.
9. Draft the IP Assignment Agreement and Partner Profit Sharing Agreement. Use a simple 2-page document — even an email chain with explicit acknowledgement is better than nothing.
10. Contact Saleem: inform him the platform is being upgraded to cloud and the subscription model is being formalised. Frame it positively — better platform, automated renewals, no more AnyDesk for routine tasks.
11. Invoice Saleem for all outstanding facility onboarding fees and set up first annual subscription invoice for 5–6 existing facilities at AED 2,500 each (minimum).
12. Prepare Channel Partner Agreement for Saleem's signature — simple 3-page document.

8.2 Development Sprint Plan (Weeks 3–12)

Week	Sprint Focus	Deliverable
Week 3–4	Phase 1 — Containerise & Deploy	Python code in Azure Function. Azure Blob for file storage. Azure SQL for facilities table. Remove Windows scheduler dependency.
Week 5–6	Phase 2 — Multi-Tenant & Credential Vault	Facilities table live. Azure Key Vault for passwords. Download job tracking. Basic Kaaryaa admin CLI.
Week 7–9	Phase 3 — Saleem Admin Portal (React)	Onboarding form, credential update, download monitor, subscription expiry view. Hosted on Azure Static Web Apps.
Week 10–11	Phase 4 — Provider Abstraction	Shafafiya calls behind HealthcareProvider interface. DHA Dubai adapter scaffold ready.
Week 12	Testing, UAT with Saleem, Production Cutover	End-to-end test with 1 live facility on cloud. Saleem completes UAT. Announce to all facilities.

8.3 Effort & Cost Estimate — Cloud Revamp

Phase	Effort (Days)	Kaaryaa Fair Rate (INR)	Recommendation
Phase 1 — Containerise & Deploy	4–5 days	INR 40,000–50,000	Absorb as platform investment (self-funded)
Phase 2 — Multi-Tenant & DB	5–6 days	INR 50,000–60,000	Absorb as platform investment
Phase 3 — Saleem Admin Portal	8–10 days	INR 80,000–1,00,000	Bill partially to Saleem as 'upgrade fee' — AED 2,000
Phase 4 — Provider Abstraction	4–5 days	INR 40,000–50,000	Absorb — strategic foundation

Phase	Effort (Days)	Kaaryaa Fair Rate (INR)	Recommendation
Phase 5 — Reporting & Facility Portal	8–10 days	INR 80,000–1,00,000	Revenue generating — bill as reporting add-on
TOTAL	~29–36 days	INR ~2.9–3.6 lakhs	Recoverable within 18 months at target subscription revenue

Self-Funding Strategy

The revamp investment of INR 2.9–3.6 lakhs can be fully self-funded from formalised subscription revenue. At AED 2,500 × 6 facilities = AED 15,000 = INR ~3.45 lakhs in Year 1 subscription revenue alone. The revamp pays for itself in subscription Year 1.

9. Product Roadmap — ASKAPI to ClaimsSync

The long-term vision is to evolve ASKAPI from a Shafafiya-specific download tool into ClaimsSync — a pan-GCC healthcare claims integration platform under the Kaaryaa GenAI Solutions brand.

Milestone	Timeline	Description
ASKAPI v1.0 — Cloud SaaS	Q2 2026	Azure-hosted, multi-tenant, Saleem admin portal. Abu Dhabi DoH (Shafafiya) only. 5–6 facilities.
ASKAPI v1.1 — Self-Service Renewals	Q3 2026	Saleem can renew licences, update passwords without Kaaryaa involvement. Automated expiry notifications.
ClaimsSync v2.0 — Dubai Module	Q3–Q4 2026	Dubai DHA API adapter added. Second channel partner in Dubai engaged. First 3 Dubai facilities onboarded.
ClaimsSync v2.1 — Facility Portal	Q4 2026	Facility manager login. Download history, claim counts, basic charts. First direct facility relationships.
ClaimsSync v2.2 — AI Anomaly Detection	Q1 2027	GenAI layer: detect unusual claim patterns, flag resubmission spikes, suggest remittance discrepancies. Kaaryaa GenAI differentiation.
ClaimsSync v3.0 — Saudi Arabia (NPHIES)	Q2–Q3 2027	Saudi NPHIES API adapter. KSA-specific compliance. Channel partner in Riyadh. 10+ facilities target.
ClaimsSync v3.x — Pan-GCC	2027–2028	Qatar, Bahrain, Kuwait, Oman adapters. SaaS platform with 50+ facilities. Potential acquisition target.

9.1 GenAI Differentiation Layer

As Kaaryaa's core identity is GenAI Solutions, the ASKAPI/ClaimsSync platform has a natural path to AI-powered features that competitors cannot easily replicate:

- Claim Pattern Analysis: AI detection of unusual rejection rates, payer-specific issues, seasonal trends
- Resubmission Intelligence: Auto-suggest corrections on failed claims based on rejection reason codes
- Remittance Reconciliation: AI matching of remittance payments to submitted claims — flags discrepancies
- Natural Language Query: 'Show me all DAMAN claims rejected in Q1 with reason code 15' — conversational BI
- Anomaly Alerts: WhatsApp or email alerts when claim download volume drops unexpectedly (facility issue detector)

Positioning Statement

ClaimsSync by Kaaryaa GenAI Solutions: The only AI-powered healthcare claims integration platform built specifically for UAE and GCC insurance providers — automating download, routing, reconciliation, and compliance reporting from Abu Dhabi DoH, Dubai DHA, and regional health authorities.

10. Summary & Priority Action Items

10.1 Three Things to Do This Week

#	Action	Owner
1	Share this document with Partners 2 and 3. Schedule a 60-minute call to align on revenue model, revamp decision, and legal agreements.	*All 3 Partners*
2	Draft IP Assignment + Profit Sharing Agreement. Even a signed email with explicit terms is a start.	*Anbu (primary drafter)*
3	Invoice Saleem for all outstanding onboarding fees + first annual subscription cycle for existing facilities at AED 2,500 minimum per facility.	*Anbu / Partner who manages Saleem relationship*

10.2 The Numbers at a Glance

Metric	Current	Target (Year 1 SaaS)	Target (Year 3)
Annual Revenue	INR ~1,07,000 (one-time)	AED 48,000 (~INR 11 lakhs)	AED 80,000+ (~INR 18.4 lakhs)
Facilities	5–6 (all via Saleem)	8–9 (Saleem + 1 direct)	20+ (multi-channel)
Markets	Abu Dhabi only	Abu Dhabi + Dubai	UAE + Saudi + Qatar
Delivery Model	Thick client .exe, AnyDesk	Cloud SaaS, Azure UAE North	Cloud SaaS, multi-region GCC
Anbu's Operational Time	High (AnyDesk, manual support)	Low (Saleem self-service portal)	Minimal (platform manages itself)
Revenue per Anbu Hour (actual)	~USD 9/hour (initial project)	USD 40–60/hour effective	USD 80+/hour effective

What Was Built Is Genuinely Valuable

A working, production-proven healthcare claims integration platform with IP ownership, a proven channel, and 5–6 live facility references.

The only thing missing is formal structure, correct pricing, and a cloud delivery model. All three are fixable. The hardest part — building it — is already done.

Document prepared by Anbu, Kaaryaa GenAI Solutions. For internal partner use only.